# Outpatient Type 2 Diabetes — Clinical One-Pager

Screen · stage complications · treat by comorbidity first, A1c second. ADA Standards of Care 2025–2026. Adapted from TeachIM.org (CC BY-NC 4.0) — clinician-reviewed before use.

## Diagnosis

| Test | Threshold |
|---|---|
| Random glucose + sx/crisis | >200 mg/dL |
| Fasting glucose (≥8 h) | ≥126 mg/dL |
| HbA1c | ≥6.5% |
| 2-h OGTT (75 g) | ≥200 mg/dL (mainly preg) |

Asymptomatic → **confirm** with repeat or 2nd method. Prediabetes: A1c 5.7–6.4%.

## Screening — who & when

- **All adults at age 35.** Earlier if BMI ≥25 (**≥23 Asian Am.**) + 1 risk factor (high-risk ethnicity, 1st-deg relative, HTN, dyslipidemia, ASCVD, PCOS, inactivity).
- Also: diabetogenic drugs (steroids, atypical antipsychotics, some HIV meds); prior gestational DM.
- Frequency: **q3 yr**; **yearly** if prediabetes; **3–6 mo** if marginal.

## A1c pitfalls

- **Falsely HIGH** — low RBC turnover: Fe/B12/folate deficiency, asplenia.
- **Falsely LOW** — high RBC turnover: hemolysis, EPO, pregnancy, CKD.
- Discordant A1c/glucose → trust fasting glucose or CGM (GMI).

## Complications

| Microvascular | Screen (annual) |
|---|---|
| Retinopathy | Dilated retinal exam |
| Nephropathy | Urine albumin:Cr (abnl >30 mg/g) |
| Neuropathy | Foot exam + monofilament |

**Macrovascular** (CAD/MI, stroke, PAD): no routine screen — evaluate by symptoms.

- Tight A1c (~7%) **reduces microvascular** dz (benefit ~10 yr).
- Macrovascular prevention = **BP, lipids, tobacco**, not glucose (legacy effect if early/new dz).
- Statin (mod-intensity primary prev); BP <130/80 if 10-yr risk >15% or ASCVD, else <140/90; smoking cessation.

## Management — comorbidity FIRST

- **Step 1:** lifestyle for all (≥5% wt loss, ≥150 min/wk) + metformin (eGFR ≥30) for most.
- **Step 2 — by comorbidity, independent of A1c:**
  - ASCVD / high risk → **GLP-1 RA ± SGLT2i**
  - HF (HFrEF or HFpEF) → **SGLT2i**
  - CKD + albuminuria → **SGLT2i** (+ ACEi/ARB)
- **Step 3 — only now, A1c:** if above goal prioritize wt loss (GLP-1 RA > SGLT2i) → cost → avoid hypoG → then SU/insulin.
- Severe (BG >300, A1c >10%, sx): start insulin up front, simplify later. Consider initial 2-drug combo if A1c 1.5–2% over goal.
- **SGLT2i & GLP-1 RA have mortality/renal benefit; insulin does not.**

## 2025–26 ADA updates

- Organ-protection (SGLT2i/GLP-1 RA) is a **core** strategy — no longer requires metformin first.
- Dual **GIP/GLP-1 RA** (tirzepatide) preferred when greater wt loss wanted (obesity, MASLD/MASH, HFpEF).
- **Finerenone** (nonsteroidal MRA) for CKD with persistent albuminuria despite ACEi/ARB + SGLT2i.

## Drug-class quick table

| Class | Watch out for |
|---|---|
| Metformin | GI upset; avoid eGFR <30 |
| GLP-1 RA | GI; pancreatitis; medullary thyroid ca CI |
| SGLT2i | GU/UTI infx; eugly DKA; volume; amputation (cana) |
| Sulfonylurea | HypoG, wt gain; **no glyburide in CKD** → glipizide |
| Insulin | HypoG, wt gain; no CV benefit |

## Glycemic targets & do/AVOID

- Most adults A1c <7%; **<8–8.5%** if frail/elderly, dementia, limited life expectancy, recurrent hypoG.
- **Do:** de-intensify insulin/SU in elders with hypoglycemia or falls.
- **AVOID:** SGLT2i after amputation/severe PAD or prior DKA; tightening A1c in frail patients.